

[Vouchcare]

PRODUCT REQUIREMENTS

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Problem

Statement:

Patients often struggle to determine the appropriate healthcare facility for their medical needs, leading to overcrowded hospitals, delayed treatment, and inefficient use of healthcare resources. At the same time, hospitals face significant delays in receiving reimbursements from Health Maintenance Organizations (HMOs), which creates cash flow challenges and can prevent patients from receiving timely care. As a result, both patients and healthcare providers experience inefficiencies that negatively impact healthcare access and service delivery.

1. Gaps in the existing product

Existing solutions tend to address only one part of the healthcare journey. Symptom checkers help users understand symptoms but don't guide them to appropriate healthcare facilities, while telemedicine platforms focus on consultations without addressing triage routing. Payment gateways process transactions but don't handle healthcare reimbursement workflows.

2. Customer Problems

Lack of reliable medical guidance to determine the appropriate level of care for their symptoms, causing Patients to visit the wrong healthcare facilities and experience delays in treatment.

At the same time, hospitals struggle with unpredictable and delayed reimbursements from HMOs which creates cash-flow uncertainty. This results in inefficient patient routing, overcrowded facilities, and hesitation from hospitals to rely on HMO payments.

3. Business & Growth Impact from a business perspective

Wrong-Tier Hospital Visits:

- Leads to overcrowding in higher-tier hospitals while primary clinics remain underutilized, leads to inefficient healthcare resource allocation.
- Drives longer waiting time and delayed treatment for critical patients, reducing overall patient satisfaction and trust in the healthcare system.
- Increases out-of-pocket spending and poor patient experience, which can discourage patients from seeking timely medical care.

Delayed Reimbursement from HMOs:

- Creates cash-flow instability for hospitals.
- Leads to hospitals reluctance in accepting HMO patients or demand upfront payments, reducing access to care.
- Limits hospitals' ability to scale operations, invest in infrastructure, and maintain quality health care delivery due to financial uncertainty.

4. Technical Challenges with Customer Impact

- Lack of data-backed triage system to match Patients' symptoms to the required facility tier, causing wasted trips and service denials.
- Absence of a real-time payment reconciliation between HMOs and Hospitals databases forces patients to wait hours for manual authorization codes before treatment can begin.
- Failure to sync specific HMO drug lists with hospital pharmacy inventory, leading to "hidden costs" when prescribed drugs are found to be uncovered at the point of dispensing.
- Lack of an automated payment rail to instantly separate "Co-pays" from "HMO portions," leads to disputes and demands for total upfront cash deposits from patients.
- Unified dashboard causes "trust friction" where hospitals refuse care because they cannot digitally track or verify the settlement status of previously rendered services.
- There is no unified "Health Path" that follows the patient from a primary clinic to a specialist hospital. This leads to patient's medical journey scattered across different clinic registers

5. Internal business problems solved

- Process efficiency

Wrong-tier referrals create massive operational waste by inflating HMO claim costs and misallocating specialist resources to primary care cases that could be treated more affordably. Simultaneously, delayed reimbursements trap hospital working capital and force administrative teams into manual reconciliation loops, shifting their focus from patient care to debt collection.

- Cost savings

Wrong tier hospital referral leads to high cost of unnecessary use of expensive specialists facilities for minor primary care cases. On the other hand, manual authorization flow increases work hours lost in manual payment reconciliation, phone-based approval and debt collection efforts. Payment delay also leads trapped working capitals and cash-flow instability

- Enabling other teams/product needs

By automating right-tier referrals, VouchCare enables Customer Support and Marketing teams to shift from managing complaints to scaling high-trust growth through predictable, reliable patient outcomes. Simultaneously, real-time reimbursement empowers Finance and Provider Management teams by eliminating manual reconciliation and resolving the payment friction that typically stunts hospital network expansion.

6. Metrics and Data use to quantify the Problem (Research-Based)

A research on Nigerian 2024-2026 healthcare data, quantifies the Healthcare Orchestration Gap by measuring the difference between clinical necessity and actual facility utilization (Tier-Mismatch), the financial latency between service delivery and settlement (Reimbursement Lag), and the resulting erosion of Patient Trust (NPS/Satisfaction). We track the 'Authorization-to-Care' timeline and the 'Claim-to-Cash' cycle to pinpoint exactly where manual HMO bottlenecks trigger hospital pushback and patient dissatisfaction.

- Quantitative Findings
 - 7.1% to 7.4% of patients arriving at tertiary (specialist) hospitals are actually referred from a primary care source.
 - 92.9% arriving patients "self-refer" or bypass the primary tier entirely.
 - **84.1%** of healthcare providers report a deficit in their operations following HMO/NHIA quarterly payment cycles.
 - Patient satisfaction among HMO enrollees is significantly lower (**~44.6%**) compared to out-of-pocket (OOP) payers in specific specialized settings, largely due to Authorization Latency.
 - Approximately **56.3%** of hospital workers explicitly cite "HMO inefficiencies" and "abysmally low" reimbursement rates as a barrier to scaling services.
- Qualitative insight
 - HMO patients often feel stigmatized and are frequently sidelined at the hospital front desk because the hospital expects payment delays.
 - Without real-time payment reconciliation, Hospital billing officers live in constant fear of "Claim Rejection."
 - Patients describe the referral process as a "blind leap" with no clear information
 - Hospital accountants spend their entire day on WhatsApp groups and phone calls begging for "authorization codes," which leads to burnout and a high turnover of administrative staff.

Solution

A multi-platform (Mobile and Desktop) orchestration layer that streamlines the healthcare value chain for existing HMO enrollees and providers.

A mobile and desktop web app that:

- Analyzes patient symptoms in real-time to generate a mandatory digital referral to the medically appropriate facility tier, preventing the overcrowding of specialist hospitals with primary care cases.
- Bridges the "Trust Gap" by instantly validating policy coverage and triggering automated split-payments between the HMO and the hospital at the point of care, eliminating the 30–90 day reimbursement hurdle.
- Consolidates fragmented referral histories, pharmacy stock availability, and financial settlement statuses into a single source of truth accessible to both hospital administrators and HMO managers.

Use cases

For the Patient (Mobile Interface)

- **As a sick enrollee, I want to** input my symptoms into the AI triage **so that** I am directed to the nearest appropriate clinic and don't waste time at a crowded specialist hospital.
- **As an HMO user, I want to** receive a digital 'Referral Token' before I leave home **so that** the hospital front desk is already expecting me and knows my plan is valid.
- **As a patient at the pharmacy, I want to** see exactly what my HMO covers in real-time **so that** I am not surprised by 'out-of-pocket' costs after the drugs are dispensed.

For the Hospital Admin (Desktop Interface)

- **As a hospital receptionist, I want to** scan a patient's digital token **so that** I can instantly verify their eligibility without making a single phone call to the HMO.
- **As a hospital accountant, I want to** see a real-time ledger of 'Guaranteed Payments' **so that** I can stop demanding cash deposits from insured patients.
- **As a medical director, I want to** receive automated settlements for services rendered **so that** I have the cash flow to restock my pharmacy and pay my staff on time.

For the HMO Manager (Desktop Interface)

- **As a** claims officer, **I want to** see AI-verified referrals in real-time **so that** I can stop manually reviewing thousands of paper claims at the end of the month.
- **As a** network manager, **I want to** track which hospitals are providing the best experience for our users **so that** I can reward high-performing partners with more volume to encourage performance.

Target User

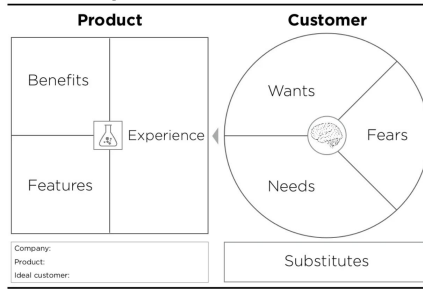
Primary problem solved

Patients	Wrong-tier hospital referral
HMO user	Long authorization delays and treatment tier denial
Hospital receptionist	Manual verification delays
Hospital accountant	Delayed, manual HMO claim reconciliations.
Medical director	Administrative friction, clinical inefficiency, degraded patient care quality
Claims officer	Overwhelming manual paperwork, high error rates and fraud
Network manager	Poor provider trust, stalled hospital expansion and partnership growth

Value Proposition

What benefit/ value will this deliver to customers? What edge do you have over competitors? Why would a customer choose this product? [Read more here](#) and download Value Prop Canvas here
We help [X] do [Y] by doing [Z]

Value Proposition Canvas



Unlike existing diagnostic AI that lacks policy context or HMOs that lack real-time guidance, Vouchcare is a high-performance web app accessible via mobile or desktop. We eliminate administrative purgatory by merging clinical triage with instant payment rails, even in low-bandwidth network zones. This transforms a fragmented search into a completed, pre-authorized, and paid-for healthcare transaction anywhere.

- Instant treatment authorization.
- Zero reimbursement delays.
- Guaranteed hospital payments.
- Correct medical tier referral.
- Eliminated manual paperwork.
- Restored patient trust

User Personas

Scenario 1

Funke, a 28-year-old HR professional with an HMO plan. Funke arrives at a hospital with a severe migraine. Usually, she waits two hours for the receptionist to "get a code" from her HMO.

Before leaving home, Funke uses the Vouchcare Web App on her phone. The AI Triage confirms her symptoms and issues a Digital Referral Token. When she arrives, the receptionist scans the code, sees the "Payment Guaranteed" status instantly, and sends her straight to the doctor.

Scenario 2

Dr. Ibrahim, a Chief Consultant at a Tertiary Teaching Hospital. Dr. Ibrahim's waiting room is packed with patients suffering from malaria and coughs—cases that should be at a local clinic. This prevents him from seeing actual surgical emergencies.

Vouchcare's Smart Routing redirects non-emergency HMO patients to accredited Primary Health Centers nearby. Dr. Ibrahim opens his Desktop Dashboard and sees a filtered list of only high-priority, correctly triaged referrals, allowing him to focus on specialized care.

Scenario 3

Meet Chidi, the Finance Manager at a private mid-sized hospital. Chidi is tired of chasing ₦5,000,000 in "pending claims" from three different HMOs. He often tells the front desk to "delay" HMO patients because the hospital lacks the cash to buy more drugs.

Chidi logs into the Vouchcare Desktop App. He sees a real-time ledger where every treated patient's payment is already "Settled" or "Escrowed." The Instant Reimbursement rail triggers a payout to the hospital wallet, giving him the liquidity to restock the pharmacy today.

Scenario 4

Mrs. Adeyemi is an Operations Head at a national HMO. She is looking at a massive bill for a patient who went to an expensive specialist for a simple flu. She has no way to prove the patient should have gone to a cheaper clinic first.

Through the Vouchcare Management Portal, she sees that 95% of her enrollees are now following AI-Triage paths. The system automatically flagged the "Wrong-Tier" attempt and guided the patient to a cost-effective clinic, saving the HMO 40% in unnecessary claim costs.

Scenario 5

Musa a trader living in a suburban area of Kaduna with spotty 2G/3G internet, needs to know which hospital will accept his HMO card without him traveling 20km only to be rejected. Most heavy medical apps won't load on his basic smartphone.

With Vouchcare, Musa can access a Light Web App optimized for low-bandwidth zones, the triage interface loads in seconds. He gets his referral code and a list of nearby "Tier-1" clinics that are currently open, saving him a wasted journey and expensive transport fare to the city.

Goals and Success criteria

Business and Operational Goal

These goals focus on the financial health and systemic efficiency of the healthcare ecosystem.

1. Reduce Medical Loss Ratio (MLR)

Shift 30% of non-emergency HMO visits from Tertiary (Specialist) hospitals to Primary/Secondary facilities within the first 12 months via AI Triage.

2. Accelerate Settlement Velocity

Reduce the average "Claim-to-Cash" cycle from 30–90 days to under 24 hours through real-time payment orchestration.

3. Decrease Administrative Overhead

Lower the man-hours spent by HMOs and Hospitals on manual verification and reconciliation by 50%.

4. Increase Provider Retention

Achieve a 90% "Trust Score" among network hospitals, measured by their willingness to accept HMO patients without demanding cash deposits.

5. Optimize Network Capacity

Balance patient distribution across the hospital network to prevent "Tier-3 Bottlenecks" and under-utilization of Primary clinics.

User-Centered Goals

1. Eliminate "Authorization Purgatory"

Reduce the patient waiting time for HMO verification at the hospital front desk to less than 2 minutes.

2. Guarantee Cost Transparency

Ensure 100% of users know their out-of-pocket responsibility (co-pays) before they enter the consultation room.

3. Maximize Accessibility

Maintain a 99.9% uptime for the web app, specifically optimized to load in under 3 seconds on 2G/Low-bandwidth networks.

4. Improve Patient NPS (Net Promoter Score)

Raise the average satisfaction rating of "HMO Patients" from the industry average of ~44% to over 80%.

5. Empower Clinical Confidence

Provide patients with a clear, AI-backed "Next Step" (Referral) so they never feel lost or rejected by the healthcare system.

User experience

UX Philosophy

The product experience is designed the realities of patients, HMO and Hospital operations:

- Weak users
- Limited technical expertise
- Inconsistent internet connectivity
- High reliance on speed and simplicity

The UX must prioritize simplicity, clarity, speed and accuracy over complexity.

Key UX Principle

1. Minimal effort, maximum clarity and accuracy

Core actions (typing symptoms, view feedback, QR code) must be achievable in seconds

2. Low learning curve

First time users should understand the product with minimal training

3. Offline experience

Critical data (like the Digital Insurance Card) is saved locally so the user can show it to a doctor even if their internet drops out inside the hospital.

1. Primary User Flow: The Patient (Mobile Web App)

This flow focuses on moving from "feeling sick" to "receiving care" with zero administrative friction.

- **Entry:** User accesses the Vouchcare URL via mobile browser (optimized for low bandwidth).
- **Authentication:** Quick login via Phone Number + OTP (One-Time Password).
- **Symptom Triage:** User interacts with the AI chat interface to describe ailments.
- **Facility Matching:** System displays authorized hospitals based on the user's specific HMO policy and the required care tier (Primary/Secondary).
- **Cost Disclosure:** User views the estimated co-pay and the amount the HMO will cover.
- **Token Generation:** User confirms the visit; the system generates a Digital Referral Token (QR Code) with a "Payment Guaranteed" status.

- **Final Action:** User presents the token at the hospital reception for instant check-in.

2. Secondary User Flow: The Hospital (Desktop Dashboard)

This flow is designed for the Receptionist and the Accountant to ensure liquidity and trust.

- **Check-In:** Receptionist scans the patient's QR code or enters the Referral ID into the desktop dashboard.
- **Instant Verification:** Dashboard displays "Green Light" status, confirming the patient's active insurance and specific authorized treatment.
- **Treatment Log:** Doctor/Nurse records the completion of service or medication dispensed.
- **Claim Trigger:** System automatically calculates the final bill based on pre-agreed HMO tariffs.
- **Payment Handshake:** Hospital receives an instant notification of "Payment Triggered"—transferring funds from the HMO escrow to the hospital's digital wallet.
- **Reconciliation:** Accountant views a daily ledger of settled claims, exported as a CSV for their internal records.

3. Secondary User Flow: The HMO (Management Portal)

This flow focuses on oversight, cost control, and automated adjudication.

- **Dashboard Overview:** Manager views real-time data on active referrals across the entire hospital network.
- **Automated Adjudication:** The system automatically approves claims that match the AI-triage and policy rules (removing manual review).
- **Fraud Detection:** AI flags any anomalies (e.g., a hospital claiming for a procedure not authorized by the initial triage).
- **Escrow Management:** Manager monitors the funding levels of the automated payment rails to ensure continuous service for enrollees.
- **Network Performance:** Manager reviews hospital "NPS" ratings and "Wait-Time" data to optimize which providers receive more referrals.

Core UX Touchpoints

Touchpoint	User Benefit
Referral Token	Instant, guaranteed treatment access.
Front-Desk Scan	Zero manual verification delays
Entry Poster	Immediate walk-in coverage transparency
Discharge Settlement	Guaranteed real-time hospital payment.

Payment Engine	Interswitch API gateway integration.
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Designs/ flows

Refer to any mockups, concepts, storyboards, user journey flows, low fidelity designs (Figma or Miro links are appropriate).

Competitor analysis

Competitors & Their offerings (Relevant to Wrong-tier hospital referral and HMO Orchestration)

Competitor	Core offering	Strengths	Weaknesses for Patients/HMO/hospitals
Reliance Health	Tech-enabled	Tech-driven speed.	Low network
Leadway Health	Self-service health coverage.	User-led approvals.	Heavy data
AXA Mansard	Omni-channel medical insurance	Massive distribution.	Manual verification.
Bastion Health	Specialized managed care	Niche specialization.	Slow authorizations.
Helium Health	Integrated hospital OS.	Deep data.	Connectivity dependent.
Curacel	Automated claims infrastructure.	Fraud detection.	Backend only.
CarePay (M-TIBA)	Digital payment exchange.	Low-cost scale.	Trust deficit.
MyCover.ai	Embedded insurance API.	Easy integration.	Non-clinical focus
Ada Health	AI-powered clinical diagnosis.	Clinical precision.	Blind context
Babylon Health	Virtual medical clinic.	Global scalability.	High overhead.
Waspito	Social telehealth network	Community engagement.	Supply dependent
Famasi	Digital pharmacy orchestration	Inventory mastery.	Logistic shocks.

Product Positioning

- The Payment Lag: Design to provide solution to verification gap most hospitals still face at the front desk where patients wait while staff manually check policy tiers.
- Bridge internet connectivity gap: Provide offline access to vital patient data/information
- The Diagnosis-Action Gap: Provide patients with detailed reference to nearby right-tier hospital according to their symptoms and their HMO plan
- The Liquidity Crisis: Provide mid-sized hospitals with tools to reconcile HMO debts

Key features and releases

Release phase	Category	Key Features	Use cases covered	Value Proposition	Launch Focus
Phase 1: MVP	Must have	Feature A: Low-Bandwidth Web Portal Loads a server- side rendered (SSR) interface using a "mobile-first, assets-last" protocol. It strips non-essential CSS/JS, prioritizing text-based logic to ensure the DOM remains interactive even	1. Connectivity-blind patient access.	Patients access premium health services without high-end hardware or expensive data plans.	Hospital receptionists and patients. Lagos urban health centers. Transaction-based verification fees. 3 month pilot launch.

		on high-latency networks.			
		Feature B AI Triage Dashboard Runs a local decision-tree algorithm that cross-references user-input symptoms against a cached HMO Policy Matrix. It maps clinical codes to specific plan exclusions without requiring a full database fetch.	2. Lack of pre-visit coverage confirmation.	Patient gets upfront knowledge of their coverage limits, reducing "billing shock" and increasing the likelihood of seeking early treatment.	
		Feature C QR Scan Generates a time-sensitive, encrypted token containing hashed patient ID and authorization status. When scanned, the app	3. Delayed identity verification.	Hospital receptionists replace 30 minutes of manual phone calls and paperwork with a 60-second scan, increasing hospital front-desk	

		executes a handshake with the provider's node to decrypt and validate the session.		throughput by up to 300%.	
		Feature D Real-Time Claims Dashboard Utilizes WebSockets to push live status updates from the HMO's approval engine directly to the hospital's UI, bypassing the need for manual Page refreshes or polling.	4. Inability to track revenue	Hospital management gets real-time "Accounts Receivable" view, replacing messy paper logs with an immutable digital record of authorized work.	
Phase 2	Should-have	Feature E: Real-Time Settlement Rail: Triggers a smart-contract or automated ledger entry upon claim adjudication. It communicates with bank APIs via a specialized	5. Delayed hospital payout	Solves the 90-day hospital debt cycle by providing 24-hour payouts, allowing clinics to stay liquid and restock life-saving supplies	Hospital accountants and HMOs. Major Nigerian tech hubs. Liquidity and commission

		bridge to initiate a "Near-Instant Transfer" to the hospital's registered settlement account.		instantly.	fees. 6-8 months financial scaling
		Feature F Co-Pay Wallet: Operates as a virtual sub-ledger. It calculates the difference between the hospital bill and the insurance "allowable amount," then executes a digital debit from the user's wallet to the hospital's balance using Interswitch Webpay API.	6. Scattered Patient payout	Consolidates insurance and out-of-pocket payments into one transaction, reducing "revenue leakage" at the hospital billing table by 15-20%.	

		Feature G Automated Adjudication: Employs a rules-based engine that compares the ICD-10 codes submitted by the doctor against the initial AI Triage data. If the difference is within pre-set parameters, the claim is auto-approved in the database.	7. Manual claim approval.	Reduces HMO administrative overhead by auto-approving low-risk, high-volume claims, allowing them to focus resources on complex medical cases.	
		Feature H Fraud Detection Engine: Runs a pattern-matching algorithm on the	8. Unaligned billing integrity verification.	Protects HMO margins by ensuring the clinical diagnosis and final hospital bill are	

		metadata of each claim (time, location, and cost-density). It flags outliers that deviate from the historical "normal" for that specific hospital or diagnosis.		aligned, significantly reducing "up-billing" and false claims.	
		Feature I Paycode for Cardless: Access By using the Interswitch Paycode API, you can generate a cardless withdrawal or payment code.	Unavailable offline pay-code	Allows the hospital to "pull" the authorized co-pay directly, matching the speed of your QR Digital Handshake	
Phase 3 Feature enhancement	Could-have	Featue J E-Pharmacy Handshake: Uses Interswitch API gateway to transmit an authorized electronic prescription from the hospital module to the pharmacy's inventory system,	9. Uninsured drug fulfillment.	Closes the gap between "getting a prescription" and "taking the medicine" by extending insurance benefits to the pharmacy counter.	Corporate employers and pharmacies. Nationwide regional expansion. Enterprise SaaS subscription model.

		verifying insurance coverage for specific drug SKUs in real-time.			12 month ecosystem integration.
		Feature K Offline Health Passport Syncs a compact, encrypted JSON file to the browser's local storage (IndexedDB). It uses a "last-known-good" sync method to ensure vital data is accessible even when the device is fully offline.	10. Lack emergency record access.	Provides critical medical data (blood group, allergies) to providers in emergency or zero-signal environments, reducing preventable medical errors.	
		Feature L Corporate Admin Portal: Aggregates multi-user data into an OLAP cube for high-speed analytical processing. It	11. Lack of access to Staff wellness analytics.	Gives employers anonymized, data-driven insights into staff wellness, enabling them to negotiate better premiums and	

		applies a differential privacy layer to ensure individual identities are masked while generating trend reports.		design more effective wellness programs.	
		Feature M Predictive Analytics: Feeds anonymized triage data into a geographic heatmap engine. It identifies statistical clusters of symptoms to detect anomalies in public health trends before they are officially reported.	12. Late or no outbreak warning.	Turns millions of triage points into a "Public Health Sentinel," allowing governments and NGOs to predict and contain outbreaks before they become crises.	

Summary

Must-Have (phase 1) Feature A - D: Low-bandwidth web portal, QR Scan, AI Policy Triage, Real-time claims dashboard

Should-Have (Phase 2) Feature E – H: Real-time settlement rail, Co-pay wallet, Fraud detection engine, Offline Health Passport

Could-Have (Phase 3) Feature I – K: E-Pharmacy handshake, Corporate admin portal, Predictive analytics, White-labeling options

Won't-Have Native mobile apps, Full clinical EMR, Direct clinical diagnosis, Hardware manufacturing.

Launch Strategy

Phase 1: Pilot web-based verification in 50 urban Lagos hospitals.

Phase 2: Deploy instant settlement rails for hospital liquidity.

Phase 3: Integrate pharmacies and corporate wellness nationwide.

User stories and requirements

Feature	User story	Requirement/s	Example UI
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1. Low-bandwidth web portal	As a patient in a low-connectivity zone, I want a lightweight web app so I can access my health data on a 2G network.	- Total page weight must be less than 2MB. - Implement Server-Side Rendering (SSR) to minimize client-side JS. - Utilize Service Workers for aggressive asset caching.	
2. QR "Digital Handshake"	As a receptionist, I want to scan a patient's code so I can instantly verify their coverage without calling the HMO.	- Generate AES-256 encrypted, time-sensitive tokens. - Include a public-key infrastructure (PKI) for offline signature validation. - Token must refresh every 60 seconds.	
3. AI Policy Triage	As an HMO enrollee, I want to see which of my symptoms are covered by my specific plan so I can avoid out-of-pocket surprises.	- Map user inputs to ICD-10 clinical codes. - Cross-reference inputs against an HMO Policy Matrix stored in a Redis cache. - Return results in <3 seconds on mobile latency.	
4. Real-time claims dashboard	As a hospital accountant, I want a live list of all authorized Vouchcare patients so I can track my daily expected revenue.	- Establish a WebSocket (WSS) connection for real-time state updates. - Implement Role-Based Access Control (RBAC) for data security. - Design a read-only mobile view for front-desk staff.	

Real-Time Settlement Rail	As a hospital medical director, I want my HMO claims settled within 24 hours so that I have the liquidity to pay staff and restock supplies	- Integration with Interswitch Transfer APIs for T+1 payouts. - Automated reconciliation between claim ID and Interswitch Transaction ID.	
Co-Pay Wallet	As a patient , I want to pay my out-of-pocket balance via a cardless paycode or web-transfer so I don't have to carry physical cash to the clinic.	- Secure hooks for Interswitch Webpay and Paycode API. - Virtual ledger to track "split" between patient and HMO funds.	

Requirements per release (optional)

Release	Feature	User Story	Requirement/s
Release 1			
Release 2			
Release 3			

Out of scope

1. EMR Hosting: No clinical record storage.
2. Medical Prescribing: No direct drug orders.
3. Hardware Manufacturing: Software only, no devices.
4. International Expansion: Focus only on Nigeria.
5. **Payment Processing:** Vouchcare does not act as a bank; all financial movement is legally and technically routed through Interswitch APIs.

Questions and decision tracker

Keep track of any open questions and decisions that need to be made. Some decisions have tradeoffs - document these here so people know that the discussions have happened and there is awareness of the tradeoffs.

Date	Question/ Decision	Answer